Referral packet F27: readable copy

Source: F27.pdf (fictional), 12 pages. Left side of each sheet is the original page, right side is a typed transcript. Black boxes are privacy redactions, not missing information.

Page map

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4	Patient face sheet	face_sheet	good	
5	History and Physical	h_and_p	good	
6	Hospitalist progress note	progress_note	good	
7	Respiratory therapy note	rt_note	good	
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Page 1: Fax cover sheet

fax_cover | orientation normal | legibility good

Sunridge Valley Medical Center (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Owen Prescott, LCSW
Phone: 555-0181
Date: 10/15/2026 08:40
Pages (including cover): 12
Re: Recuperative care referral
Urgent **[X]** For Review **[X]** Please Reply **[X]**

Message

34F asthma exacerbation, resolved. Independent, walking halls.
Hoping for d/c to recuperative care 10/15 PM. Please call with questions.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

FAX F27 FICTIONAL DEMO RECORD · NOT A REAL PATIENT PAGE 02/12

ILLUMINATION Recuperative Care Referral Form

HEALTH • HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ilfhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Sunridge Valley Medical Center (fictional) Dept/Floor: 4 East Medical
SW/CM/RN: Owen Prescott, LCSW Phone/Pager #: 555-0181
Email: oprescott@sunridge.example Nursing Station #: 555-0182
Authorized by: Dr. M. Castellano Phone #:

Patient Information

Patient Name: Angela Doe MRN: SV-2093516 Date of Birth (dd/mm/yyy): 02/03/1992
Gender: ☐ Male ☒ Female ☐ Transgender ☐ Other Ethnicity: Black or African American
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Acute asthma exacerbation
Admit Date: 10/12/2026 Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 10/12/2026 Results: CXR - hyperinflation, no infiltrate

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	✕ Yes* No	Active Infection	Yes* ✕ No
*Explain 2L nasal cannula		*Explain	
Self Administer All Meds	✕ Yes No*	Ambulatory	✕ Yes No
Continent of Bowel & Bladder	✕ Yes No	Assistive Device	Yes* ✕ No
Colostomy/Ileostomy	Yes ✕ No	*Explain	
Foley Catheter	Yes ✕ No	Hospice	Yes* ✕ No
Diabetes	Yes ✕ No	*Explain	
Insulin	Yes ✕ No	Car	Yes ✕ No
Diabetes related Oral Meds	Yes ✕ No	Service Animal	Yes ✕ No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? ✕ No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain None

Length of Stay Authorized in Recupertive Care: 10 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

Page 2: IHH Recuperative Care Referral Form

referral_form | orientation normal | legibility good

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Sunridge Valley Medical Center (fictional)
Department/Floor: 4 East Medical
Social Worker/Case Manager/Nurse: Owen Prescott, LCSW
Phone: 555-0181
Email: oprescott@sunridge.example
Nursing Station: 555-0182
Authorized By: Dr. M. Castellano

Patient Information

Name: Angela Doe
MRN: SV-2093516
Birth Date: 02/03/1992
Gender: F
Ethnicity: Black or African American
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: English
Housing Situation:
Currently Unhoused **[X]**
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Acute asthma exacerbation
Admit Date: 10/12/2026
Any Wounds: Yes [ ] No **[X]**
Number/Location/Size/Stage:
Independent with Wound Care: Yes [ ] No [ ]
Home Health Ordered: Yes [ ] No **[X]**
PPD/TB Test or Chest X-Ray: Date: 10/12/2026 Result: CXR - hyperinflation, no infiltrate

History of Health & Living Situation

O2: Yes **[X]** No [ ] 2L nasal cannula
Self Administer All Meds: Yes **[X]** No [ ]
Continent of Bowel & Bladder: Yes **[X]** No [ ]
Colostomy/Ileostomy: Yes [ ] No **[X]**
Foley Catheter: Yes [ ] No **[X]**
Diabetes: Yes [ ] No **[X]**
Insulin: Yes [ ] No **[X]**
Diabetes Oral Meds: Yes [ ] No **[X]**
Active Infection: Yes [ ] No **[X]**
Ambulatory: Yes **[X]** No [ ]
Assistive Device: Yes [ ] No **[X]**
Hospice: Yes [ ] No **[X]**
Car: Yes [ ] No **[X]**
Service Animal: Yes [ ] No **[X]**

Behavioral and Mental Health Status

Any limitations or challenges: No **[X]** Yes [ ]
Mental Health DX: Yes [ ] No **[X]** Type:
Non-compliant: Yes [ ] No **[X]**
Cognitive Impairment: Yes [ ] No **[X]**
Forgetful: Yes [ ] No **[X]**
Registered Sex Offender: Yes [ ] No **[X]**
Other: None

Length of Stay Authorized

Days: 10

Illumination Staff Only

Date Received:
Reviewed By:
Approved: Yes [ ] No [ ]
Date:
Authorization #:
Reason for Denial:
Admission Date/Time:
Exit Date:
Extension Request: Yes [ ] No [ ]

Referring Party Comments

O2 2L NC per admission orders.
Independent with ADLs. Needs stable place to finish steroid taper.

-- FICTIONAL DEMO RECORD - not a real patient --



FAX F27 FICTIONAL DEMO RECORD - NOT A REAL PATIENT PAGE 03/12

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST
Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal
Member
Member Name: Angela Doe
DOB: 02/03/1992
Member ID: OCH-55841177
Referring Provider
Facility: Sunridge Valley Medical Center (fictional)
Clinical
Primary Diagnosis: Asthma, moderate persistent, with acute exacerbation
ICD-10: J45.41
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 10/15/2026
Requested Days: 10
Admit Date: 10/12/2026
Clinical summary: asthma exacerbation resolved, room air, independent ADLs.
Plan Use Only
Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 3: CalAIM Community Supports authorization request

calaim_auth | orientation normal | legibility good

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Angela Doe
DOB: 02/03/1992
Member ID: OCH-55841177

Referring Provider

Facility: Sunridge Valley Medical Center (fictional)

Clinical

Primary Diagnosis: Asthma, moderate persistent, with acute exacerbation
ICD-10: J45.41
Member is experiencing homelessness **[X]**
Member at risk of hospitalization/re-hospitalization **[X]**
Requested Start Date: 10/15/2026
Requested Days: 10
Admit Date: 10/12/2026
Clinical summary: asthma exacerbation resolved, room air, independent ADLs.

Plan Use Only

Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Patient face sheet

Sunridge Valley Medical Center (fictional) - PATIENT FACE SHEET
Patient: Angela Doe
MRN: SV-2093516
Account #: 99093516
DOB: 02/03/1992
Sex: Female
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)
Encounter
Admit Date/Time: 10/12/2026 03:10 (ED arrival 10/11/2026 23:45)
Admit Type: Inpatient
Unit/Bed: 4E-421
Attending: Miguel Castellano, MD
Insurance
Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self
Emergency Contact
None listed

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 4: Patient face sheet

face_sheet | orientation normal | legibility good

Sunridge Valley Medical Center (fictional) - PATIENT FACE SHEET

Patient: Angela Doe
MRN: SV-2093516
Account #: 99093516
DOB: 02/03/1992
Sex: Female
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 10/12/2026 03:10 (ED arrival 10/11/2026 23:45)
Admit Type: Inpatient
Unit/Bed: 4E-421
Attending: Miguel Castellano, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical

HISTORY AND PHYSICAL
Date: 10/12/2026
CC
Wheezing and shortness of breath x 2 days
HPI
34 y/o F with moderate persistent asthma, unhoused x 1 yr (sleeps in her car until it was towed 3 weeks ago, now outdoors), presents with wheezing and dyspnea after wildfire smoke exposure and running out of her controller inhaler. In ED SpO2 90% RA, given continuous albuterol nebs, IV methylprednisolone, magnesium. Placed on 2L NC.
No prior intubations. No ICU admissions.
PMH
- Asthma since childhood
- Seasonal allergic rhinitis
Meds PTA
Albuterol inhaler PRN (ran out); fluticasone-salmeterol (ran out 1 month ago)
Allergies
Penicillin (rash)
Social
Never smoker. Rare EtOH. Denies drug use.
Exam
T 98.6F HR 112 BP 132/84 RR 26 SpO2 93% 2L NC. Diffuse expiratory wheezes. Speaking in short sentences. AOx4.
Assessment/Plan
1. Acute asthma exacerbation - albuterol/ipratropium nebs q4h, methylprednisolone 40 mg IV q12h then prednisone taper, O2 to SpO2 >= 92%
2. Dispo - SW consult (unhoused)
Signed: Miguel Castellano, MD 10/12/2026 05:20

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 5: History and Physical

h_and_p | orientation normal | legibility good

HISTORY AND PHYSICAL

Date: 10/12/2026

CC

Wheezing and shortness of breath x 2 days

HPI

34 y/o F with moderate persistent asthma, unhoused x 1 yr (sleeps in her car until it was towed 3 weeks ago, now outdoors), presents with wheezing and dyspnea after wildfire smoke exposure and running out of her controller inhaler. In ED SpO2 90% RA, given continuous albuterol nebs, IV methylprednisolone, magnesium. Placed on 2L NC.
No prior intubations. No ICU admissions.

PMH

- Asthma since childhood
- Seasonal allergic rhinitis

Meds PTA

Albuterol inhaler PRN (ran out); fluticasone-salmeterol (ran out 1 month ago)

Allergies

Penicillin (rash)

Social

Never smoker. Rare EtOH. Denies drug use.

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Assessment/Plan

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2. Dispo - SW consult (unhoused)
Signed: Miguel Castellano, MD 10/12/2026 05:20

-- FICTIONAL DEMO RECORD - not a real patient --

Hospitalist progress note

HOSPITALIST PROGRESS NOTE - HD#4
Date: 10/15/2026 07:30
S: Breathing back to baseline. No nighttime symptoms. Walking halls without dyspnea.
O: T 98.2F HR 78 BP 118/72 RR 16 SpO2 96% RA. Clear to auscultation, no wheezes. AOx4. Peak flow 380 L/min (personal best approx 400).
A/P:
1. Asthma exacerbation - resolved. Off nebulizers since 10/13. On room air since 10/13 10:00. Prednisone 40 mg PO daily, last dose 10/17. Restart fluticasone-salmeterol BID; albuterol MDI PRN. No antibiotics (no infection).
2. Dispo - medically stable for d/c to recuperative care today. Pulmonology f/u in 2 wks.
Signed: Miguel Castellano, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Input Lab demo. Names, facilities and numbers are fictional.

Page 6: Hospitalist progress note

progress_note | orientation normal | legibility good

HOSPITALIST PROGRESS NOTE - HD#4

Date: 10/15/2026 07:30
S: Breathing back to baseline. No nighttime symptoms. Walking halls without dyspnea.
O: T 98.2F HR 78 BP 118/72 RR 16 SpO2 96% RA. Clear to auscultation, no wheezes. AOx4. Peak flow 380 L/min (personal best approx 400).
A/P:
1. Asthma exacerbation - resolved. Off nebulizers since 10/13. On room air since 10/13 10:00. Prednisone 40 mg PO daily, last dose 10/17. Restart fluticasone-salmeterol BID; albuterol MDI PRN. No antibiotics (no infection).
2. Dispo - medically stable for d/c to recuperative care today. Pulmonology f/u in 2 wks.
Signed: Miguel Castellano, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Respiratory therapy note

RESPIRATORY THERAPY NOTE
Date: 10/15/2026 09:00
O2 weaned off 10/13 10:00; on room air since. Ambulatory oximetry 10/14: rest SpO2 97% RA, after walking 250 ft SpO2 96% RA.
Nebulizer treatments discontinued 10/13. No breathing treatments needed in last 48 hours other than her own MDI PRN (used once 10/14).
Inhaler technique taught with spacer; pt demonstrated correct technique with teach-back. Asthma action plan reviewed.
No home O2 indicated.
Signed: D. Ferreira, RRT

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

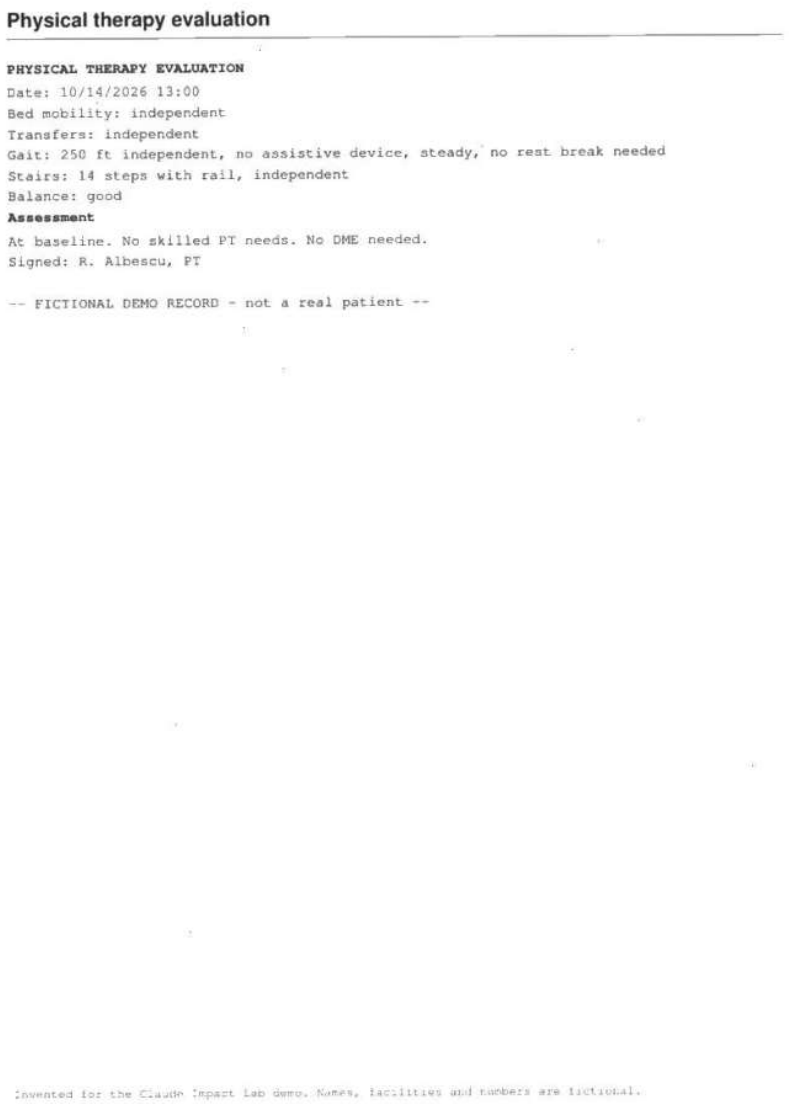
Page 7: Respiratory therapy note

rt_note | orientation normal | legibility good

RESPIRATORY THERAPY NOTE

Date: 10/15/2026 09:00
O2 weaned off 10/13 10:00; on room air since. Ambulatory oximetry 10/14: rest SpO2 97% RA, after walking 250 ft SpO2 96% RA.
Nebulizer treatments discontinued 10/13. No breathing treatments needed in last 48 hours other than her own MDI PRN (used once 10/14).
Inhaler technique taught with spacer; pt demonstrated correct technique with teach-back. Asthma action plan reviewed.
No home O2 indicated.
Signed: D. Ferreira, RRT

-- FICTIONAL DEMO RECORD - not a real patient --



Physical therapy evaluation

PHYSICAL THERAPY EVALUATION
Date: 10/14/2026 13:00
Bed mobility: independent
Transfers: independent
Gait: 250 ft independent, no assistive device, steady, no rest break needed
Stairs: 14 steps with rail, independent
Balance: good
Assessment
At baseline. No skilled PT needs. No DME needed.
Signed: R. Albescu, PT

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 8: Physical therapy evaluation

pt_eval | orientation normal | legibility good

PHYSICAL THERAPY EVALUATION

Date: 10/14/2026 13:00
Bed mobility: independent
Transfers: independent
Gait: 250 ft independent, no assistive device, steady, no rest break needed
Stairs: 14 steps with rail, independent
Balance: good

Assessment

At baseline. No skilled PT needs. No DME needed.
Signed: R. Albescu, PT

-- FICTIONAL DEMO RECORD - not a real patient --

Nursing shift note

NURSING SHIFT NOTE
Date: 10/15/2026 06:00
Pt AOx4. Independent with all ADLs. Showered independently 10/14 PM. Continent of bowel and bladder, toilets independently.
Self-administers prednisone and inhalers, teach-back done.
Calm and cooperative. No behavioral concerns this admission.
Isolation status: none (standard precautions).
Room air, SpO2 96%. Afebrile.
Signed: T. Nakashima, RN

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

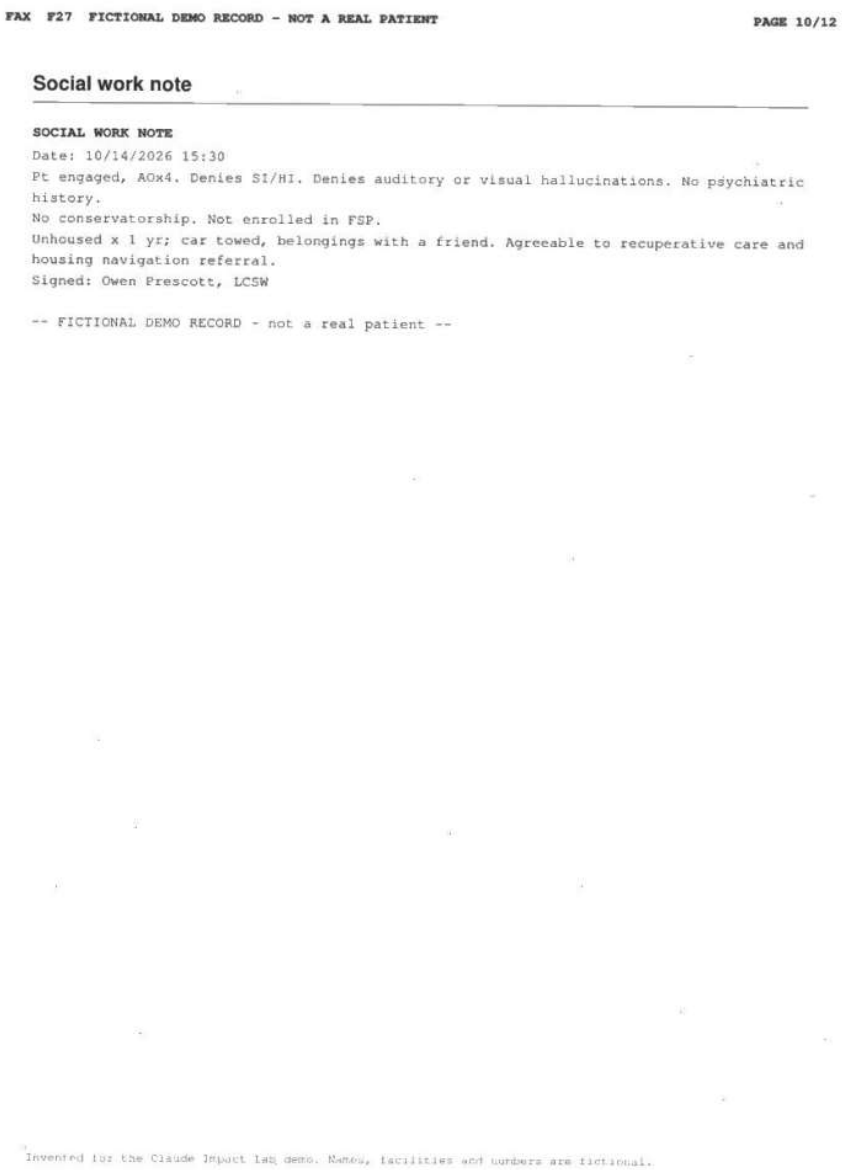
Page 9: Nursing shift note

nursing_note | orientation normal | legibility good

NURSING SHIFT NOTE

Date: 10/15/2026 06:00
Pt AOx4. Independent with all ADLs. Showered independently 10/14 PM. Continent of bowel and bladder, toilets independently.
Self-administers prednisone and inhalers, teach-back done.
Calm and cooperative. No behavioral concerns this admission.
Isolation status: none (standard precautions).
Room air, SpO2 96%. Afebrile.
Signed: T. Nakashima, RN

-- FICTIONAL DEMO RECORD - not a real patient --



Page 10: Social work note

social_work | orientation normal | legibility good

SOCIAL WORK NOTE

Date: 10/14/2026 15:30
Pt engaged, AOx4. Denies SI/HI. Denies auditory or visual hallucinations. No psychiatric history.
No conservatorship. Not enrolled in FSP.
Unhoused x 1 yr; car towed, belongings with a friend. Agreeable to recuperative care and housing navigation referral.
Signed: Owen Prescott, LCSW

-- FICTIONAL DEMO RECORD - not a real patient --

Labs, imaging and vitals

LABS

10/12 | WBC 11.8 H | Eos 6% | Hgb 13.2 | Plt 265 | Na 138 | K 3.6 | Cr 0.7 | Glucose 142
10/14 | WBC 10.9 | K 3.9 | Glucose 118
10/12 Respiratory viral panel (incl. SARS-CoV-2 PCR): Negative
10/12 MRSA nares PCR: Negative
10/15 05:50 SARS-CoV-2 rapid antigen: Negative

IMAGING

10/12 CXR PA/lat: Hyperinflation. No infiltrate, no effusion, no pneumothorax.

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
10/14 08:00	98.4	82	120/74	16	97%	RA
10/14 14:00	98.6	80	116/70	16	96%	RA
10/14 20:00	98.2	76	118/72	14	97%	RA
10/15 02:00	98.0	74	114/70	14	96%	RA
10/15 07:30	98.2	78	118/72	16	96%	RA

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 11: Labs, imaging and vitals

labs | orientation normal | legibility good

LABS

10/12 | WBC 11.8 H | Eos 6% | Hgb 13.2 | Plt 265 | Na 138 | K 3.6 | Cr 0.7 | Glucose 142
10/14 | WBC 10.9 | K 3.9 | Glucose 118
10/12 Respiratory viral panel (incl. SARS-CoV-2 PCR): Negative
10/12 MRSA nares PCR: Negative
10/15 05:50 SARS-CoV-2 rapid antigen: Negative

IMAGING

10/12 CXR PA/lat: Hyperinflation. No infiltrate, no effusion, no pneumothorax.

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
10/14 08:00 | 98.4 | 82 | 120/74 | 16 | 97% | RA
10/14 14:00 | 98.6 | 80 | 116/70 | 16 | 96% | RA
10/14 20:00 | 98.2 | 76 | 118/72 | 14 | 97% | RA
10/15 02:00 | 98.0 | 74 | 114/70 | 14 | 96% | RA
10/15 07:30 | 98.2 | 78 | 118/72 | 16 | 96% | RA

-- FICTIONAL DEMO RECORD - not a real patient --

Discharge medication list

DISCHARGE MEDICATION LIST
Patient: Angela Doe MRN: SV-2093516
- Prednisone 40 mg PO daily through 10/17/2026 (2 doses remaining)
- Fluticasone-salmeterol 250/50 1 inhalation BID
- Albuterol HFA 2 puffs q4h PRN wheeze, with spacer
- Cetirizine 10 mg PO daily
No IV medications. No home O2. No antibiotics.
Patient able to self-administer all medications: Yes
Meds-to-beds: 30-day supply of all medications dispensed at discharge

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 12: Discharge medication list

med_list | orientation normal | legibility good

DISCHARGE MEDICATION LIST

Patient: Angela Doe MRN: SV-2093516

- Prednisone 40 mg PO daily through 10/17/2026 (2 doses remaining)
- Fluticasone-salmeterol 250/50 1 inhalation BID
- Albuterol HFA 2 puffs q4h PRN wheeze, with spacer
- Cetirizine 10 mg PO daily

No IV medications. No home O2. No antibiotics.

Patient able to self-administer all medications: Yes

Meds-to-beds: 30-day supply of all medications dispensed at discharge

-- FICTIONAL DEMO RECORD - not a real patient --